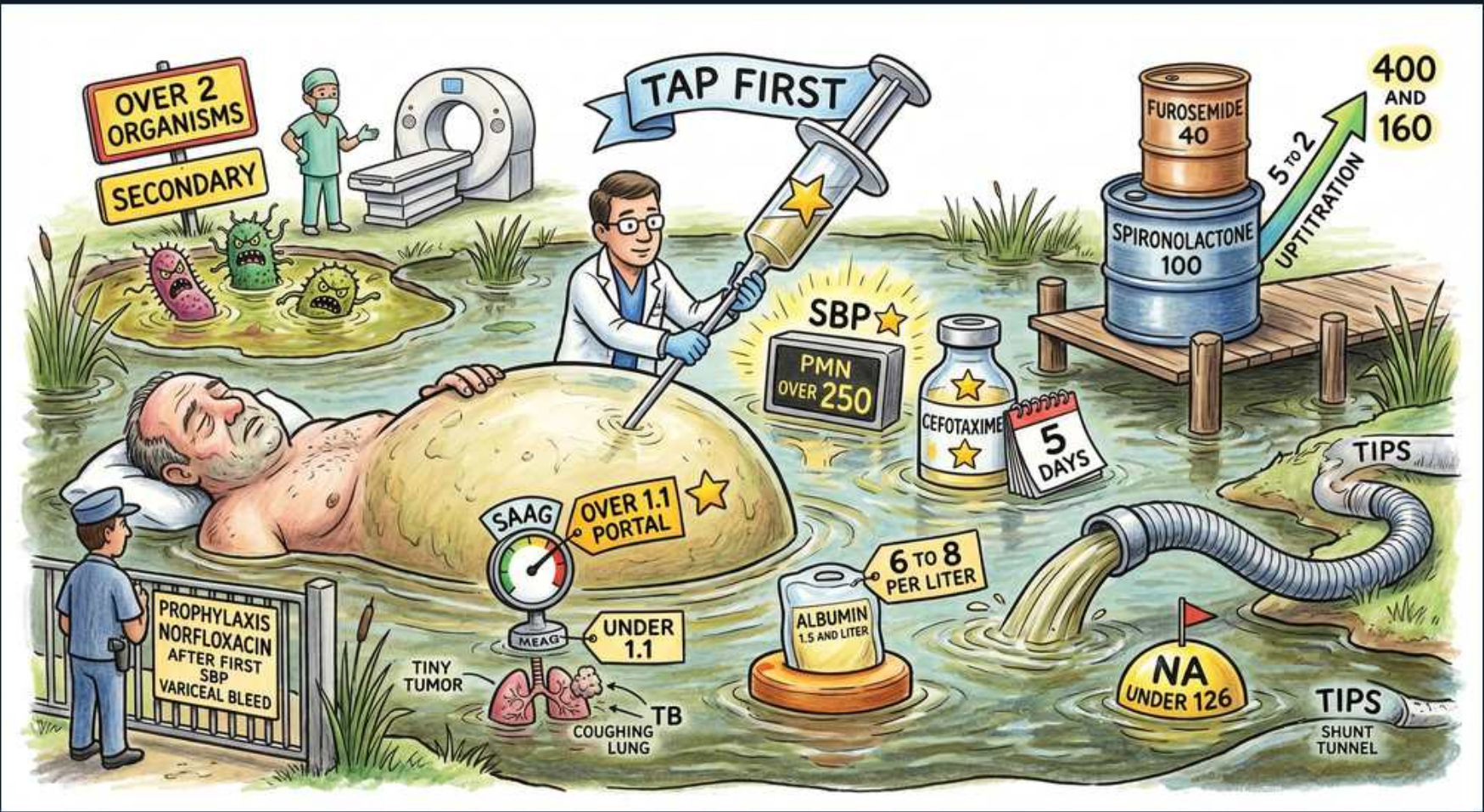


Cirrhosis — SBP & Ascites



1. Tap first (gold star)

WHAT YOU SEE

A large banner 'TAP FIRST' with a syringe and gold star, top-center.

WHAT IT ENCODES

Any NEW ascites or any cirrhotic admitted/decompensated gets a diagnostic paracentesis FIRST (gold star) before treatment. The tap gives cell count, SAAG, culture, and protein, which drive every downstream decision. Do not start diuretics or antibiotics blindly. Paracentesis is safe even with elevated INR.

2. PMN >250 = SBP (gold star)

WHAT YOU SEE

A black sign 'SBP — PMN OVER 250' with a gold star, center.

WHAT IT ENCODES

Spontaneous bacterial peritonitis is diagnosed by ascitic PMN (neutrophil) count ≥ 250 cells/mm³ (gold star), regardless of culture result. Usually monomicrobial (E. coli, Klebsiella, Strep). Treat empirically immediately on this threshold; don't wait for culture.

3. Cefotaxime 5 days

WHAT YOU SEE

A vial labeled 'CEFOTAXIME' beside a calendar '5 DAYS', center-right.

WHAT IT ENCODES

First-line SBP therapy is a third-generation cephalosporin — cefotaxime 2 g IV q8h (or ceftriaxone) for 5 days. Recheck paracentesis at 48h if not improving (expect $>25\%$ PMN drop). Use piperacillin-tazobactam/carbapenem for nosocomial or recent prophylaxis.

4. Albumin (prevents HRS)

WHAT YOU SEE

A bottle 'ALBUMIN 1.5 G AND LITER' with '6 TO 8 PER LITER' banner, center-low.

WHAT IT ENCODES

In SBP give IV albumin 1.5 g/kg on day 1 and 1.0 g/kg on day 3 — it expands volume and PREVENTS hepatorenal syndrome, reducing mortality. Omitting albumin is a tested error. For large-volume paracentesis (>5 L) give albumin 6-8 g per liter removed to prevent post-paracentesis circulatory dysfunction.

BOARD TRAP

"Treat SBP with antibiotics alone, no albumin" → wrong; omitting albumin (1.5 g/kg day 1, 1 g/kg day 3) risks hepatorenal syndrome and higher mortality.

5. SAAG >1.1 = portal (gold star)

WHAT YOU SEE

A gauge 'SAAG — OVER 1.1 PORTAL' with gold star, center-left.

WHAT IT ENCODES

Serum-ascites albumin gradient ≥ 1.1 g/dL indicates portal hypertension (cirrhosis, heart failure, Budd-Chiari) — gold star. SAAG = serum albumin minus ascitic albumin. High SAAG narrows the differential to portal causes.

6. SAAG <1.1 = non-portal

WHAT YOU SEE

An 'UNDER 1.1' tag with a tiny tumor and 'coughing lung' (TB), lower-left.

WHAT IT ENCODES

SAAG <1.1 g/dL = non-portal ascites: peritoneal carcinomatosis (tumor), TB peritonitis, pancreatic, nephrotic. These have HIGH ascitic protein. Work-up shifts to cytology, ADA/AFB for TB, imaging — not diuretics.

7. >2 organisms = secondary (X)

WHAT YOU SEE

Road signs 'OVER 2 ORGANISMS' and 'SECONDARY' with a surgeon and CT scanner, top-left.

WHAT IT ENCODES

Polymicrobial ascites (>2 organisms), very high PMN, low glucose, high LDH, or high protein suggests SECONDARY bacterial peritonitis (perforation/abscess) — needs CT imaging and SURGERY plus broadened antibiotics, NOT simple SBP management. Treating it as plain SBP misses a surgical emergency.

BOARD TRAP

"Polymicrobial / >2 organism peritonitis is just SBP — give cefotaxime and observe" → wrong; that pattern is secondary peritonitis requiring CT and surgical source control.

8. Diuretics 100:40 (5:2)

WHAT YOU SEE

Stacked tanks 'FUROSEMIDE 40' over 'SPIRONOLACTONE 100' with '5 to 2' arrow, top-right.

WHAT IT ENCODES

Ascites diuretic regimen = spironolactone 100 mg : furosemide 40 mg (the 100:40, 5:2 ratio) to keep normokalemia. Uptitrate together to a max 400:160 mg/day. Spironolactone alone is fine for mild ascites; add furosemide for potency and K⁺ balance. Combine with sodium restriction.

9. Na <126 (hyponatremia)

WHAT YOU SEE

A sign 'NA UNDER 126' near a draining pipe, lower-right.

WHAT IT ENCODES

Watch for dilutional hyponatremia (Na <126) with aggressive diuresis; hold/reduce diuretics and restrict free water. Severe hyponatremia is associated with worse outcomes and is a flag in MELD-Na. Avoid rapid correction.

10. Refractory ascites → TIPS

WHAT YOU SEE

A pipe/tunnel labeled 'TIPS — SHUNT TUNNEL', right side.

WHAT IT ENCODES

Refractory ascites (no response to max diuretics + salt restriction, or diuretic-intolerant) is managed with serial large-volume paracentesis + albumin, then TIPS for durable control; transplant evaluation. TIPS reduces ascites but risks encephalopathy.

11. Prophylactic norfloxacin

WHAT YOU SEE

A guard at a gate 'PROPHYLAXIS NORFLOXACIN AFTER FIRST SBP / VARICEAL BLEED', lower-left.

WHAT IT ENCODES

Long-term oral norfloxacin (or ciprofloxacin) prophylaxis is indicated after a first SBP episode (secondary), during acute variceal bleeding, or with low ascitic protein (<1.5 g/dL) plus renal/liver dysfunction (primary). It reduces SBP recurrence and mortality.
